IMPERIAL COLLEGE HEALTHCARE NHS TRUST

Directorate of Clinical Imaging & Diagnostic Radiology
St Mary's Hospital, Praed Street, Paddington, London W2 1NY
Radiology Appointments & PACs Desk: 020 3312 6666 | Radiology Results Desk: 020 3312 2145
Electronic Radiology Ordering: rad-orders.smh@nhs.net | Trust ODS: RYJ01

FORMAL DIAGNOSTIC RADIOLOGY REPORT — PLAIN FILM (X-RAY)

EXAMINATION: RADIOGRAPH OF CHEST (PA ERECT SINGLE VIEW)

Conforming to Royal College of Radiologists (RCR) Standards for Electronic Interpretation & Verification

Patient Full Name: Mr David Arthur PENDLETON
NHS Number: 481 920 6351
Hospital ID (MRN): SMH-774019-B
Date of Birth: 04 October 1974 (Age: 51)
Sex / Gender: Male
Patient Location: Urgent Treatment Centre (UTC)

Date of Examination: 08 September 2026
Time of Acquisition: 11:24 BST
Time of Authorisation: 13:40 BST
Accession Number: ACC-XR-2026-99120
Radiology Event ID: RAD-EV-8830114
Order Modality Code: NICIP: CR010 (Plain CXR)

Referring Clinician: Dr Fiona Campbell (GMC: 7109432)
Referring Service: UTC / Ambulatory Care Unit
Lead Radiographer: Ms S. Jenkins (HCPC: RA49102)

Reporting Consultant: Dr Neil Armstrong, FRCR
Radiologist GMC No: 6098234
Radiation Dose (DAP): 0.08 dGy·cm^2

Clinical Indication & Referral Reason

Indication stated on electronic order request: 51-year-old male presenting with a 3-week history of worsening productive cough with mucopurulent sputum, low-grade subjective pyrexia, right-sided pleuritic chest discomfort, and mild breathlessness on exertion. Non-smoker. Auscultation revealed focal bronchial breathing and coarse crackles over the right lower zone. Sepsis score low (NEWS2 = 1). Please evaluate for focal consolidation, underlying parenchymal lesion, or pleural effusion.

Imaging Technique & Radiographic Quality

- **Projection:** Standard Erect Posteroanterior (PA) chest radiograph acquired at 1.8 metres focal-film distance.
- **Image Quality / Technical Adequacy:** Good inspiratory volume achieved with 8 posterior ribs visible above the right hemidiaphragmatic dome. Scapulae are satisfactorily rotated clear of the lung fields. Nil patient motion artefact. Adequate penetration with lower thoracic vertebral bodies faintly perceptible behind the cardiac silhouette.

Prior Imaging Comparison

- Compared with previous routine occupational chest radiograph dated **14 November 2024** (Image Accession: ACC-XR-2024-11842). The baseline film was completely clear without focal pathology.

Detailed Anatomical Findings

Radiological Impression & Diagnostic Coding

1. **Acute Right Lower Lobe Consolidation:** In keeping with community-acquired alveolar pneumonia (CAP) in the appropriate clinical setting.

Anatomical Zone	Radiological Observations
Lungs & Airspaces	There is an ill-defined, non-segmental patch of airspace opacification identified in the right lower zone, overlying the right lower lobe. Air bronchograms are faintly visible within the opacity. The right hemidiaphragm contour remains crisp and visible, confirming that the consolidation is predominantly posterior basal rather than middle lobe (no silhouette sign with the right heart border). The left lung field is completely clear and fully aerated with normal bronchovascular markings throughout. There are no discrete pulmonary masses, solitary nodules, or apical cavitation identified.
Pleura	Both costophrenic angles and cardiophrenic sulci are sharp and clear. There is no blunting to suggest a pleural effusion or empyema. No pneumothorax is identified on either side.
Cardiomediastinum	The cardiothoracic ratio is normal (< 0.50). The aortic arch, aortopulmonary window, and hilar contours are within normal limits. Trachea is midline. No mediastinal shift or widening.
Bones & Soft Tissues	The visualised thoracic skeleton (clavicles, ribs, sternum, and visualized vertebrae) shows normal cortical continuity and bone density with no acute fracture or suspicious osteolytic/osteoblastic lesions. Soft tissues of the chest wall are symmetric and unremarkable.
Upper Abdomen	No abnormal free subdiaphragmatic gas (pneumoperitoneum) beneath either hemidiaphragmatic leaflet. Gastric fundus air bubble is normal.

2. **No Complications:** Nil radiological evidence of parapneumonic effusion, parenchymal cavitation, pulmonary oedema, or pneumothorax.
3. **ICD-10 Diagnostic Code: J18.9** (Pneumonia, unspecified organism) **/ J18.1** (Lobar pneumonia).
4. **SNOMED CT Code: 233604007** (Pneumonia) | **RadLex: RID4949** (Consolidation).

Radiologist Recommendations & Safety Netting

- **Clinical Correlation:** Recommend targeted antimicrobial therapy and supportive management in accordance with local trust antimicrobial guidelines and CRB-65 assessment.
- **Follow-up Radiograph (BTS Guidelines):** In accordance with the British Thoracic Society (BTS) guidelines for the management of community-acquired pneumonia, a repeat chest radiograph is recommended in **6 weeks** post-treatment to verify complete radiological resolution and confidently exclude an underlying endobronchial or parenchymal lesion.

Electronic Verification & Digital Signature:

Dr Neil Armstrong, MBChB, MRCP, FRCR
Consultant Diagnostic Radiologist
General Medical Council (GMC) Number: 6098234
Department of Clinical Radiology, St Mary's Hospital, London
Verified via Picture Archiving and Communication System (PACS) at 13:40 BST